

India's Trusted Health Test @Home Service



NABL
ACCREDITED



Booking ID : 17861486134

Sample Collection Date : 18/Apr/2026

Sunita Nigam

Female, 47 Yrs

A Comprehensive Health Analysis Report

AI Based Personalized Report for You



INDIA'S FIRST & ONLY CREDIBILITY CHECK FOR YOUR LAB REPORT

Check the authenticity of your lab report with machine data

Scan the QR using any QR code scanner

MACHINE & QC DETAILS

LABORATORY EQUIPMENT DASHBOARD

Sunita Nigam

Booking ID : 17861486134



Beckman AU700

Beckman Coulter : Beckman AU700

Department - Biochemistry

No. of test(s) performed - 38

Performed Test(s): RA Test Rheumatoid Arthritis Factor, Quantitative, CRP (C Reactive Protein) Quantitative, Serum, HsCRP High Sensitivity CRP, Liver Function Test, Iron Studies, Kidney Function Test (KFT), Lipid Profile

Description: The Beckman Coulter state-of-the-art clinical chemistry analyzer manufactured in USA, delivers exceptional performance with a throughput of up to 800 tests per hour using spectrophotometry and 1,200 tests per hour with ISE, ensuring outstanding diagnostic accuracy and precision. Utilizing advanced spectrophotometric technology, it provides reliable and highly accurate results, supporting over 21 parameters that meet Six Sigma performance standards for superior laboratory efficiency and quality.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Siemens Centaur XP

Siemens Healthineers : Siemens Centaur XP

Department - Immunology

No. of test(s) performed - 5

Performed Test(s): Vitamin B12 Cyanocobalamin, Vitamin D Total-25 Hydroxy, Thyroid Profile-Total (T3, T4 & TSH Ultra-sensitive)

Description: The Siemens Healthineers state-of-the-art immunoassay analyzer manufactured in Germany uses direct chemiluminescence with Acridinium Ester technology to deliver highly sensitive testing with a throughput of up to 240 tests per hour.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Tosoh G8

Tosoh : Tosoh G8

Department - BIOCHEMISTRY HBA1C

No. of test(s) performed - 2

Performed Test(s): HbA1c

Description: The Tosoh Corporation fully automated HbA1c analyzer manufactured in Japan uses gold-standard cation exchange technology for direct, accurate, and precise HbA1c measurement. It is NGSP certified, IFCC/DCCT traceable, and unaffected by common haemoglobin variants.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Horiba Yumi H550EE

HORIBA : Horiba Yumi H550EE

Department - Hematology

No. of test(s) performed - 24

Performed Test(s): COMPLETE BLOOD COUNT

Description: The fully automated DIFF + ESR analyzer manufactured in France, utilizes i-Double Diff and CORA ESR technologies, with CORA providing ESR results through advanced Correlated Rouleaux Analysis of RBC aggregation for precise and reliable hematology testing.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)

HEALTH ANALYSIS

Personalized Summary & Vital Parameters

Sunita Nigam

Booking ID : 17861486134 | Sample Collection Date : 18/Apr/2026

Sunita Nigam,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

Your Health Score

84
Out of 100

*Calculated from test reports



Thyroid Function

Thyroid Stimulating Hormone (TSH) – Ultrasensitive : 3.73 μ IU/ml

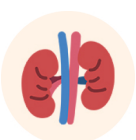
● Everything looks good



Cholesterol Total

163.1 mg/dl

● Everything looks good



Kidney Function

Serum Creatinine : 0.7 mg/dl

● Everything looks good



Vitamin D

19.84 ng/ml

● Concern



HbA1c

5 %

● Everything looks good



Vitamin B12

66 pg/ml

● Concern



Liver Function

Alanine Aminotransferase (ALT/SGPT) : 18.7 U/L

● Everything looks good



Calcium Total

9.3 mg/dl

● Everything looks good



Iron studies

Serum Iron : 72.6 μ g/dl

● Everything looks good



Complete Hemogram

Haemoglobin (HB) : 12.9 g/dL

● Everything looks good





New Features

Report Summary

Sunita Nigam

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Understanding laboratory reports can be complex, often leading to unwarranted anxiety.

At Healthians, we understand that you shouldn't have to rely on a Google search to decipher your own health report. That's why we offer comprehensive summaries that are easy to understand.

Here is a summary and suggestions based on your recent health test results:

Summary of Your Health Test Results

Hello Sunita, thank you for sharing your recent health test results. I've reviewed them and want to offer some reassurance. It's common to find a few areas that are outside the typical range, and these results provide valuable information that can help us focus on your well-being. Many of these findings are quite manageable and can be addressed with lifestyle adjustments and targeted support. We'll work together to understand these results better and create a plan that supports your health.

Suggestions for Your Health

- Vitamin D and B12 Support:** Your results indicate lower levels of Vitamin D and Vitamin B12. These vitamins are crucial for energy, mood, and overall bodily functions. Increasing your intake through dietary sources rich in these nutrients and considering appropriate supplementation can be very beneficial.
- Addressing Inflammation:** The high-sensitivity C-reactive protein (HsCRP) result suggests a level of inflammation in your body. While this can be influenced by many factors, focusing on an anti-inflammatory diet, managing stress, and ensuring adequate sleep can help reduce inflammation.
- Optimizing Blood Counts:** Several of your blood count parameters are outside the normal range, including variations in red blood cell size and volume, and certain white blood cell counts. These can sometimes be linked to nutritional status or other factors. Ensuring a balanced diet with adequate iron, B vitamins, and other essential nutrients is important for healthy blood cell production.
- Lifestyle and Dietary Review:** Given the various findings, a comprehensive review of your diet and lifestyle habits would be a good next step. This includes ensuring you are consuming a well-rounded diet, staying hydrated, getting regular physical activity, and managing stress effectively. These foundational elements play a significant role in many of the areas highlighted in your results.

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Age/Gender	: 47Y 0M 0D /Female	Sample Collected On	: 18/Apr/2026 11:39AM	
Order Id	: 17861486134	Sample Received On	: 18/Apr/2026 01:07PM	
Referred By	: Self	Report Generated On	: 18/Apr/2026 02:34PM	
Customer Since	: 18/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: EDTA Plasma	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY HBA1C

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Test Name	Value	Unit	Bio. Ref Interval
HbA1c - Glycosylated Hemoglobin			
Hba1c (Glycosylated Hemoglobin)	5.00	%	4.2 - 5.7
Method: HPLC			
Machine: TOSOH G8			
Average Estimated Glucose	96.80	mg/dl	
Method: Calculated			

INTERPRETATION:

AS PER AMERICAN DIABETES ASSOCIATION (ADA):

REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7
5.7 – 6.4
≥ 6.5
Age > 19 Years
Goals of Therapy: < 7.0
Actions Suggested: >8.0
Age < 19 Years
Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
 - HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
 - Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapson, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
 - HbA1c may be increased in patients with polycythemia or post-splenectomy.
 - Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
 - Trends in HbA1c are a better indicator of diabetic control than a solitary test.
 - Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
 - HbA1c target in pregnancy is to attain level <6 % .
 - HbA1c target in paediatric age group is to attain level < 7.5 %.
- Method : Ion-exchange high-performance liquid chromatography (HPLC).
Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023

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Referred By	: Self	Report Generated On	: 18/Apr/2026 01:52PM	
Customer Since	: 18/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Sodium Fluoride	Report Status	: Final Report	

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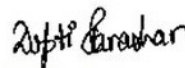
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Test Name	Value	Unit	Bio. Ref Interval
Fasting Blood Sugar			
Glucose, Fasting	80.98	mg/dl	70 - 100
Method: Hexokinase			
Machine: BECKMAN COULTER AU 700			

American Diabetes Association Reference Range :

Normal : < 100 mg/dl
 Impaired fasting glucose(Prediabetes) : 100 - 126 mg/dl
 Diabetes : >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include: Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance), Chronic kidney disease, Cushing syndrome, Excessive consumption of food, Hyperthyroidism, Pancreatitis
 A low level of glucose may indicate hypoglycemia, a condition characterized by a drop in blood glucose to a level where first it causes nervous system symptoms (sweating, palpitations, hunger, trembling, and anxiety), then begins to affect the brain (causing confusion, hallucinations, blurred vision, and sometimes even coma and death). A low blood glucose level (hypoglycemia) may be seen with: Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease, Hypopituitarism, Hypothyroidism, Severe infections, Severe heart failure, Chronic kidney (renal) failure, Insulin overdose, Tumors that produce insulin (insulinomas), Starvation.



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Customer Since	: 18/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
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Rheumatoid Factor (RA) - Quantitative - Serum

RHEUMATOID FACTOR	<10	IU/mL	<14
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Method: Immunoturbidimetric

Machine: BECKMAN COULTER AU 700

The rheumatoid factor (RF) test is primarily used to help diagnose rheumatoid arthritis (RA) and to help distinguish RA from other forms of arthritis or other conditions that cause similar symptoms. A cyclic citrullinated peptide (CCP) antibody test can help diagnose RA in someone who has joint inflammation with symptoms that suggest but do not yet meet the criteria of RA and may be ordered along with RF or if the RF result is negative. The RF test must be interpreted in conjunction with a person's symptoms and clinical history. A negative RF test does not rule out RA.

About 20% of people with RA will have very low levels of or no detectable RF. In these cases, a CCP antibody test may be positive and used to confirm RA. Positive RF test results may also be seen in 1-5% of healthy people and in some people with conditions such as: Sjogren syndrome, scleroderma, systemic lupus erythematosus (lupus), sarcoidosis, endocarditis, tuberculosis, syphilis, HIV/AIDS, hepatitis, infectious mononucleosis, cancers such as leukemia and multiple myeloma, parasitic infection, or disease of the liver, lung or kidney.

C-Reactive Protein (CRP) -Quantitative

C-REACTIVE PROTEIN (CRP)	4.82	mg/L	<5
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(QUANTITATIVE)

Method: Immunoturbidimetric

Machine: BECKMAN COULTER AU 700

C-reactive protein (CRP) is one of the most sensitive acute-phase reactants for inflammation. Measuring changes in the concentration of CRP provides useful diagnostic information about the level of acuity and severity of a disease. Unlike ESR, CRP levels are not influenced by hematologic conditions such as anemia, polycythemia etc.

Increased levels are consistent with an acute inflammatory process. After onset of an acute phase response, the serum CRP concentration rises rapidly (within 6-12 hours and peaks at 24-48 hours) and extensively. Concentrations above 100 mg/L are associated with severe stimuli such as major trauma and severe infection (sepsis).

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Sample Type	: Serum	Report Status	: Final Report	

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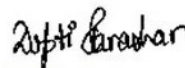
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Test Name	Value	Unit	Bio. Ref Interval
hs CRP (C-Reactive Protein high sensitive)			
HS-CRP (HIGH SENSITIVITY C-REACTIVE PROTEIN)	4.00	mg/L	< 1.0
Method: Immunoturbidimetric			
Machine: BECKMAN COULTER AU 700			

High sensitivity C-reactive protein, when used in conjunction with other clinical laboratory evaluation of acute coronary syndromes, may be useful as an independent marker of prognosis for recurrent events in patients with stable coronary disease or acute coronary syndrome. Hs-CRP levels should not be substituted for assessment of traditional cardiovascular risk factors. Patients with persistently unexplained, marked elevation of hs-CRP after repeated testing should be evaluated for non-cardiovascular etiologies.

Clinical significance :

Hs-CRP measurements may be used as an independent risk marker for the identification of individuals at risk for future cardiovascular disease. Elevated CRP values may be indicative of prognosis of individuals with acute coronary syndromes, and may be useful in the management of such individuals.



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Sample Type	: Serum	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
Liver Function Test (LFT)			
Serum Bilirubin, (Total)	0.60	mg/dl	0.3 - 1.2
Method: Diazonium Ion			
Machine: BECKMAN COULTER AU 700			
Serum Bilirubin, (Direct)	0.10	mg/dl	0 - 0.2
Method: Diazotization			
Machine: BECKMAN COULTER AU 700			
Serum Bilirubin, (Indirect)	0.50	mg/dl	0.0 - 0.8
Method: Calculated			
Aspartate Aminotransferase (AST/SGOT)	23.20	U/L	3 - 35
Method: UV without P5P			
Machine: BECKMAN COULTER AU 700			
Alanine Aminotransferase (ALT/SGPT)	18.7	U/L	3 - 35
Method: UV Without P5P			
Machine: BECKMAN COULTER AU 700			
Alkaline Phosphatase (ALP)	69.50	U/L	33-98
Method: PNPP,AMP Buffer			
Machine: BECKMAN COULTER AU 700			
Gamma Glutamyl Transferase (GGT)	13.7	U/L	5- 38
Method: Modified IFCC-L-Glutamyl-3-Carboxy-4-Nitroanilide			
Machine: BECKMAN COULTER AU 700			
Serum Total Protein	7.00	gm/dl	6.6 - 8.3
Method: Biuret Method			
Machine: BECKMAN COULTER AU 700			
Serum Albumin	3.90	g/dl	3.5 - 5.2
Method: BCG			
Machine: BECKMAN COULTER AU 700			
Serum Globulin	3.10	gm/dl	3.0 - 4.2
Method: Calculated			
Albumin/Globulin Ratio	1.26	Ratio	1.2 - 2.5
Method: Calculated			
SGOT/SGPT Ratio	1.24	Ratio	0.7 - 1.4
Method: Calculated			

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels results from increased bilirubin production (eg hemolysis and ineffective erythropoiesis); decreased bilirubin excretion (eg; obstruction and hepatitis); and abnormal bilirubin metabolism (eg; hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in viral hepatitis; drug reactions, alcoholic liver disease conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion

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Customer Since	: 18/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
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reaction & a common metabolic condition termed Gilbert syndrome.

AST levels increase in viral hepatitis, blockage of the bile duct, cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. Ast levels may also increase after a heart attack or strenuous activity. ALT is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Paget's disease, Rickets, Sarcoidosis etc.

Elevated serum GGT activity can be found in diseases of the liver, Biliary system and pancreas. Conditions that increase serum GGT are obstructive liver disease, high alcohol consumption and use of enzyme-including drugs etc.

Serum total protein, also known as total protein, is a biochemical test for measuring the total amount of protein in serum. Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma, Waldenström's disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic - Human serum albumin is the most abundant protein in human blood plasma. It is produced in the liver. Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc. Please note that there is a change in the Biological Reference Interval for Alkaline Phosphatase (ALP) parameter. (Dated: 12.02.2026).

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Customer Since	: 18/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: SERUM	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
Iron study			
Serum Iron	72.6	ug/dl	60 - 180
Method: TPTZ-No Deproteinization			
Machine: BECKMAN COULTER AU 700			
UIBC	285.70	ug/dl	155 - 355
Method: Nitroso-PSAP			
Machine: BECKMAN COULTER AU 700			
Serum Total Iron Binding Capacity (TIBC)	358.3	µg/dl	250 - 400
Method: FE+UIBC (saturation with iron)			
Transferrin Saturation %	20.26	%	15 - 50
Method: Calculated			

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells. It is a constituent of the oxygen-carrying chromoproteins, haemoglobin and myoglobin, as well as various enzymes, such as cytochrome oxidase and peroxidases.

Serum iron may be increased in hemolytic, megaloblastic and aplastic anemias, and in hemochromatosis acute leukemia, lead poisoning, pyridoxine deficiency, thalassemia, excessive iron therapy, and after repeated transfusions. Drugs causing increased serum iron include chloramphenicol, cisplatin, estrogens (including oral contraceptives), ethanol, iron dextran, and methotrexate. Iron can be decreased in iron-deficiency anemia, acute and chronic infections, carcinoma, nephrotic syndrome hypothyroidism, in protein- calorie malnutrition and after surgery. Diurnal variation is seen in serum iron levels with normal values obtained in the midmorning, low values in midafternoon and very low values near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Transferrin saturation represents the number of iron-binding sites that are occupied. It is a better index of iron stores than serum iron alone. Transferrin saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).

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Sample Type	: Serum	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
(KFT) Kidney Function Test			
Serum Creatinine Method: Alkaline Picrate Method Machine: BECKMAN COULTER AU 700	0.70	mg/dl	0.4 - 0.9
GFR, ESTIMATED Method: Calculated (CKD-EPI)	107.28	mL/min/1.73m2	
Serum Uric Acid Method: Uricase Machine: BECKMAN COULTER AU 700	4.9	mg/dl	2.6-6.0
Serum Calcium Method: ARSENAZO III Machine: BECKMAN COULTER AU 700	9.3	mg/dl	8.8 - 10.6
Serum Sodium Method: ISE (Indirect) Machine: BECKMAN COULTER AU 700	139	mmol/L	136 - 146
Serum Chloride Method: ISE Indirect Machine: BECKMAN COULTER AU 700	104	mmol/L	101 - 109
Blood Urea Method: Urease ,UV Machine: BECKMAN COULTER AU 700	19.1	mg/dl	17 - 43
Blood Urea Nitrogen (BUN) Method: Calculated	8.9	mg/dl	8-20
Bun/Creatinine Ratio Method: Calculated	12.76	Ratio	
Urea/Creatinine Ratio	27.29		

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.

Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle.

Uric Acid is the major product of purine catabolism in humans. Uric acid levels are used to monitor the treatment of gout.

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DEPARTMENT OF BIOCHEMISTRY

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Test Name **Value** **Unit** **Bio. Ref Interval**

Measurement of calcium is used in the diagnosis and treatment of parathyroid disease, a variety of bone diseases, chronic renal disease, urolithiasis and tetany. Phosphorus levels are increased in acute or chronic renal failure with decreased GFR .

Sodium is an electrolyte, and it helps regulate the amount of water in and around the cells & the balance of chemicals in the body called acids and bases. Potassium is a primary intracellular ion, only 2 % is extracellular, high intracellular concentration is maintained by a Na- K ATPase pump, which continuously transports potassium into the cell against a concentration gradient. Chloride is a negatively charged ion that works with other electrolytes such as potassium, sodium, and bicarbonate, to help regulate the amount of fluid in the body and maintain the acid-base balance.

EGFR-National kidney Disease Education program recommends the use of MDRD equation to predict/ estimate GFR in adults (>=20 years) with Chronic kidney disease. Calculated by serum creatinine may be less accurate due to certain factor like race muscle mass, diet and certain drugs in such case EGFR should be calculated using serum cystatin C.

Age (Years)	Male ML/min/1.73m ²	Female ML/min/1.73m ²
17-24	93-131	—
25-34	78-146	—
35-44	74-138	65-123
45-54	74-129	58-110
55-64	69-122	50-110
65-74	61-114	46-105
75-84	52-102	48-85

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UPMC Reg. No-97570, Healthians Labs



SIN No:E7811730

Patient Name	: Sunita Nigam	Barcode	: E7811730	
Age/Gender	: 47Y 0M 0D /Female	Sample Collected On	: 18/Apr/2026 11:39AM	
Order Id	: 17861486134	Sample Received On	: 18/Apr/2026 01:12PM	
Referred By	: Self	Report Generated On	: 18/Apr/2026 02:02PM	
Customer Since	: 18/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

One Plus One Super Life Gold Advance

Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile			
Total Cholesterol Method: CHO/POD Machine: BECKMAN COULTER AU 700	163.1	mg/dl	Desirable : <200 Borderline: 200-239 High : >=240
Serum Triglycerides Method: GPO-POD Machine: BECKMAN COULTER AU 700	101.8	mg/dl	Desirable : <150 Borderline high : 150-199 High : 200-499 Very high : > 500
Serum HDL Cholesterol Method: Enzymatic Immunoinhibition (CHO-POD) Machine: BECKMAN COULTER AU 700	44.3	mg/dl	40 - 60
LDL Cholesterol Calculated Method: Calculated	98.44	mg/dl	Optimal : <100 near /above Optimal:100 - 129 Borderline High: 130- 159 High : 160 - 189 Very High :>=190
VLDL Cholesterol Calculated Method: Calculated	20.36	mg/dl	<30
Total CHOL / HDL Cholesterol Ratio Method: Calculated	3.68	Ratio	3.30 - 4.40
LDL / HDL Cholesterol Ratio Method: Calculated	2.22	Ratio	Desirable/Low Risk: 0.5-3.0 Line/Moderate Risk: 3.0-6.0 Elevated/High Risk: >6.0
HDL / LDL Cholesterol Ratio Method: Calculated	0.45	Ratio	Optimal->0.4 Moderate-0.4 to 0.3 High-<0.3
Non-HDL Cholesterol Method: Calculated	118.8	mg/dl	0.0 - 160.0

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency. Dyslipidemias means increase in the level of one or more of the following: Total Cholesterol, low density lipoprotein (LDL) and/or triglyceride concentrations. Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood. Cholesterol is a steroid carried in the bloodstream as lipoprotein, necessary for cell membrane functioning and as a precursor to bile acids, progesterone, vitamin D, estrogens, glucocorticoids and mineralocorticoids. HDL is termed "good cholesterol" because its levels are inversely related to the risk of Coronary heart disease.

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Test Name	Value	Unit	Bio. Ref Interval
LDL cholesterol is termed the "bad cholesterol" and their increased levels are associated with increased risk of atherosclerosis and coronary heart disease. Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. Healthians labs report biological reference intervals (normal ranges) in accordance with the recommendations of The National Cholesterol Education Program (NCEP) & Adult Treatment Panel IV (ATP IV) guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal lipid levels.			

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Referred By	: Self	Report Generated On	: 18/Apr/2026 03:31PM	
Customer Since	: 18/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

One Plus One Super Life Gold Advance

Test Name	Value	Unit	Bio. Ref Interval
Urine Routine & Microscopy Extended			
PHYSICAL EXAMINATION			
Colour	Pale Yellow		Pale Yellow
Method: Visual			
Volume	25.00	mL	
Method: Visual			
Appearance	Clear		Clear
Method: Visual			
CHEMICAL EXAMINATION			
Specific Gravity	1.020		1.001 - 1.035
Method: Dipstick-Ion exchange			
Machine: URIPLUS 600 NAME			
pH	6.0		4.5 - 7.5
Method: Dipstick-Double indicator			
Machine: URIPLUS 600 NAME			
Glucose	Negative		Negative
Method: Dipstick-oxidase peroxidase			
Machine: URIPLUS 600 NAME			
Urine Protein	Negative		Negative
Method: Dipstick-Bromophenol blue			
Machine: URIPLUS 600 NAME			
Ketones	Negative		Negative
Method: Sodium nitroprusside			
Machine: URIPLUS 600 NAME			
Urobilinogen	Normal		Normal
Method: Dipstick-Ehrlichs Test			
Machine: URIPLUS 600 NAME			
Bilirubin	Negative		Negative
Method: Dipstick-Ehrlichs Test			
Machine: URIPLUS 600 NAME			
Nitrite	Negative		Negative
Method: Dipstick-Griess test			
Machine: URIPLUS 600 NAME			
Blood	Nil		Nil
Method: Dipstick-Peroxidase			
Leucocyte Esterase	Negative		Nil

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DEPARTMENT OF CLINICAL PATHOLOGY

One Plus One Super Life Gold Advance

Test Name	Value	Unit	Bio. Ref Interval
Method: Dipstick- Esterase			
Machine: URIPLUS 600 NAME			
MICROSCOPIC EXAMINATION			
Pus Cells	1-2	/HPF	0 - 5
Method: Microscopic Examination			
Epithelial cells	1-2	/HPF	0 - 5
Method: Microscopic Examination			
RBCs	Nil	/HPF	Nil
Method: Microscopic Examination			
Casts	Nil		Nil
Method: Microscopic Examination			
Crystals	Nil		Nil
Method: Microscopic Examination			
Bacteria	Absent		Absent
Method: Microscopic Examination			
Yeast Cell	Absent		
Others (Non Specific)	Nil		
Method: Microscopy			

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Order Id	: 17861486134	Sample Received On	: 18/Apr/2026 01:07PM	
Referred By	: Self	Report Generated On	: 18/Apr/2026 03:42PM	
Customer Since	: 18/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

One Plus One Super Life Gold Advance

Test Name	Value	Unit	Bio. Ref Interval
Complete Blood Count			
Haemoglobin (HB)	12.9	g/dL	12.0-15.0
Method: Spectrophotometry			
Machine: Yumizen H550E			
Total Leucocyte Count (TLC)	6.7	10 ³ /uL	4.0-10.0
Method: Impedance			
Machine: Yumizen H550E			
Hematocrit (PCV)	39.4	%	36.0-46.0
Method: Calculated			
Machine: Yumizen H550E			
Red Blood Cell Count (RBC)	3.76	10 ⁶ /μl	3.80-4.80
Method: Impedance			
Machine: Yumizen H550E			
Mean Corp Volume (MCV)	104.6	fL	83.0-101.0
Method: Derived from RBC Histogram			
Machine: Yumizen H550E			
Mean Corp Hb (MCH)	34.4	pg	27.0-32.0
Method: Calculated			
Machine: Yumizen H550E			
Mean Corp Hb Conc (MCHC)	32.9	g/dL	31.5-34.5
Method: Calculated			
Machine: Yumizen H550E			
RDW - CV	14.2	%	11.6-14.0
Method: Derived from RBC Histogram			
Machine: Yumizen H550E			
RDW - SD	59.90	fL	39.0-46.0
Method: Derived from RBC Histogram			
Machine: Yumizen H550E			
Mentzer Index	27.82	Ratio	
Method: Calculated			
RDWI	395.03	Ratio	
Method: Calculated			
Green and king index	120	Ratio	
Method: Calculated			
Differential Leucocyte Count			
Neutrophils	39.3	%	40 - 80
Method: Double Hydrodynamic Sequential System DHSS			

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DEPARTMENT OF HAEMATOLOGY

One Plus One Super Life Gold Advance

Test Name	Value	Unit	Bio. Ref Interval
Machine: Yumizen H550E			
Lymphocytes	55.1	%	20-40
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Monocytes	4.8	%	02 - 10
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Eosinophils	0.7	%	01 - 06
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Basophils	0.1	%	00 - 02
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC)	2.64	10 ³ /uL	2.0-7.0
Method: Calculated			
Machine: Yumizen H550E			
Absolute Lymphocyte Count (ALC)	3.71	10 ³ /uL	1.0-3.0
Method: Calculated			
Machine: Yumizen H550E			
Absolute Monocyte Count	0.32	10 ³ /uL	0.2-1.0
Method: Calculated			
Machine: Yumizen H550E			
Absolute Eosinophil Count (AEC)	0.05	10 ³ /uL	0.02-0.5
Method: Calculated			
Machine: Yumizen H550E			
Absolute Basophil Count	0.01	10 ³ /uL	0.02 - 0.10
Method: Calculated			
Machine: Yumizen H550E			
Platelet Count(PLT)	163	10 ³ /μl	150-410
Method: Impedance			
Machine: Yumizen H550E			
MPV	9.7	fL	7 - 9
Method: Derived from PLT Histogram			
Machine: Yumizen H550E			

Platelets, TLC and DLC reviewed on smear

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making.

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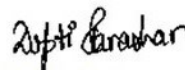
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DEPARTMENT OF HAEMATOLOGY

One Plus One Super Life Gold Advance

Test Name	Value	Unit	Bio. Ref Interval
This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.			

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely causes, making it necessary to distinguish between them. If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron-deficiency anaemia is more likely.



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Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

One Plus One Super Life Gold Advance

Test Name	Value	Unit	Bio. Ref Interval
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Vitamin B12

VITAMIN B12	66	pg/ml	211 - 912
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Method: Chemiluminescence (CLIA)

Machine: SIEMENS CENTAUR XP

Vitamin B12 is a coenzyme that is involved in two very important metabolic functions vital to normal cell growth and DNA synthesis: 1) the synthesis of methionine, and 2) the conversion of methylmalonyl CoA to succinyl CoA. Deficiency of this vitamin can lead to megaloblastic anemia and ultimately to severe neurological problems. Also causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency.

Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	19.84	ng/ml	30 - 100
-------------------------------	-------	-------	----------

Method: Chemiluminescence (CLIA)

Machine: SIEMENS CENTAUR XP

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL), Adult	VITAMIN D 25 HYDROXY (ng/mL), Pediatric
DEFICIENCY	<20	<15
INSUFFICIENCY	20 - 30	15 - 20
SUFFICIENCY	30 - 100	20 - 100

Vitamin D is a lipid-soluble steroid hormone that is produced in the skin through the action of sunlight or is obtained from dietary sources. The role of vitamin D in maintaining homeostasis of calcium and phosphorus is well established.

The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).

The reference ranges discussed in the preceding are related to total 25-OHD; as long as the combined total is 30 ng/mL or more, the patient has sufficient vitamin D.

Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20-30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL.

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DEPARTMENT OF IMMUNOLOGY

One Plus One Super Life Gold Advance

Test Name	Value	Unit	Bio. Ref Interval
Thyroid Profile (Total T3, T4, TSH)			
Tri-Iodothyronine (T3, Total)	1.27	ng/ml	0.60-1.81
Method: Chemiluminescence (CLIA)			
Machine: SIEMENS CENTAUR XP			
Thyroxine (T4, Total)	7.50	ug/dl	3.2-12.6
Method: Chemiluminescence (CLIA)			
Machine: SIEMENS CENTAUR XP			
Thyroid Stimulating Hormone (TSH)-Ultrasensitive	3.730	μIU/ml	0.55-4.78
Method: Chemiluminescence (CLIA)			
Machine: SIEMENS CENTAUR XP			

Pregnancy interval	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

Healthians recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
9. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

*** End Of Report ***

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SIN No:E7811730

Terms & Conditions:

- 1) Machine Data is available for last 7 days only. In case of manual testing & outsourced testing, machine data will not be available.
- 2) CBC parameters may vary when it is manually reviewed by the Pathologists.
- 3) **For Thyroid tests** - Circulating TSH shows a normal circadian rhythm with a peak between 11pm-5am and a nadir between 5pm-8pm. TSH values are also lowered after food when compared to fasting in a statistically significant manner. This variation is of the order of $\pm 50\%$, hence time of day and fasting status have influence on the reported TSH level.
- 4) **For Lipid profile** - Lipid and Lipoprotein concentrations vary during the normal course of daily activity. Also, certain drugs, diet and alcohol can have lasting effects on Triglyceride levels. To obtain best results for Lipid testing, a strict fasting of 10-12 hours with a light meal on the previous night is recommended.
- 5) Test results released pertain to the specimen submitted.
- 6) Test results are dependent on the quality of the sample received by the Lab.
- 7) The tests are carried out in the lab with the presumption that the specimen belongs to the patient named or identified in the bill/test request form/booking ID.
- 8) The reported results are for information and are subject to confirmation and interpretation by the referring doctor to co-relate clinically.
- 9) Test results may show interlaboratory variations.
- 10) Liability of Healthians for deficiency of services or other errors and omissions shall be limited to the fee paid by the patient for the relevant laboratory services.
- 11) This report is not subject to use for any medico-legal purposes.
- 12) Few of the tests might be outsourced to partner labs as and when required.
- 13) This report is not intended to replace but to lead by providing comprehensive information. It is recommended that you consult your doctor/physician for interpretation of results.
- 14) All reports might not be applicable for individuals less than 18, pregnant women or individuals suffering from diseases for which health test has not been performed or symptoms not diagnosed.
- 15) This report is based on preventive health test screening and is meant for a healthy lifestyle. It does not provide any recommendation for life threatening situations.
- 16) It is strongly recommended to take required precautions for allergic reactions or sensitivities.
- 17) Authorised partner labs as mentioned for certain tests are as below:
 - HL/PL/001- Metropolis Healthcare Ltd
 - HL/PL/002- Thyrocare technologies Limited
 - HL/PL/003- Lifecell International Pvt. Ltd. - Laboratory Services
 - HL/PL/004- Modern Diagnostic & Research Centre

ADVISORY

Health Advisory

Sunita Nigam

Booking ID : 17861486134 | Sample Collection Date : 18/Apr/2026

28.44 Body Mass Index

4'11" Height (ft/in)

64 Weight (kgs.)



Physical Activity

No Data



Smoke

No Data



Food Preference

No Data



Alcohol

No Data



Medication

No Data



Family History

No Data



Blood Pressure

No Data



Pulse Rate

No Data



Waist (In Cm)

No Data



Hip Circumference (In Cm)

No Data



SPO2 Levels

No Data



Sugar Levels

No Data

Additional Remarks :

NA

SUGGESTED NUTRITION

SUGGESTED NUTRITION

Do's

- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Take vitamin C rich foods like citrus fruits, strawberries and green, leafy vegetables
- Include nuts like almonds, walnuts and seeds like flaxseeds, sunflower seeds
- Have vitamin B12 fortified cereal and dairy.
- Include fruits like apples, berries and melons in your diet
- Include calcium rich foods like milk, yoghurt, cheese and green, leafy vegetables
- Include Brazil nuts, sesame seeds, sunflower seeds

Dont's

- Avoid red meat and organ meats
- Avoid refined sugars, processed foods and bakery items
- Reduce caffeine intake
- Limit sugar intake
- Avoid high cholesterol and calorie dense foods
- Avoid the use of oil and avoid sauces and dressings
- Avoid flavoured and seasoned foods
- Decrease intake of colas and sugary drinks
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods

SUGGESTED LIFESTYLE

SUGGESTED LIFESTYLE

Do's

- Stay active and maintain ideal weight
- Have regular exposure to sunlight

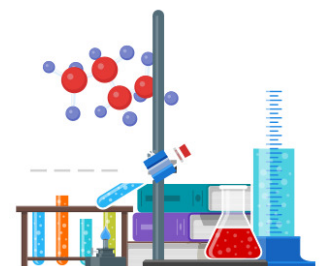
Dont's

- Avoid overworking or being stressed for long time
- Avoid having long gaps in meals or skipping meals
- Avoid smoking and alcohol
- Don't ignore your body signals and don't skip your regular health check-ups
- Avoid strenuous exercises
- Avoid overexertion without having food or drink

SUGGESTED FUTURE TESTS

SUGGESTED FUTURE TESTS

- Vitamin B12 Cyanocobalamin - **Every 1 Month**
- Vitamin D Total-25 Hydroxy - **Every 1 Month**
- Folic Acid - **Every 1 Month**
- Electrocardiogram - **Every 1 Month**
- CPK, Total - **Every 1 Month**
- LH-Luteinizing Hormone - **Every 1 Month**
- Lipid Profile Advance - **Every 1 Month**



HEALTH ADVISORY

Suggestions for Health & Well-being

Sunita Nigam

Booking ID : 17861486134 | Sample Collection Date : 18/Apr/2026

PHYSICAL ACTIVITY

PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week.

If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet.

It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

BALANCED DIET

STRESS MANAGEMENT

STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



BMI

BMI recommended range is 18.5 to 24.9. Your BMI is 29.35, which is on a higher side.

BMI INFORMATION NOT AVAILABLE
Please fill your Health Karma to know your BMI results
BMI for your body helps prevent many untimely diseases and goes a long way.

BMI CHART

UNDERWEIGHT	NORMAL	OVERWEIGHT	OBSE
Less than 18.5	Between 18.5 - 24.9	Between 25.0 - 29.9	More than 30
*****	*****	*****	*****

BMI

Supplement Suggestions

Sunita Nigam

Booking ID : 17861486134 | Sample Collection Date : 18/Apr/2026

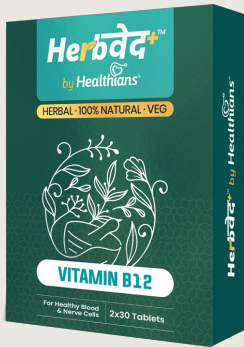
Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Vitamin B12 Cyanocobalamin	VITAMIN B12
Vitamin D Total-25 Hydroxy	VITAMIN D3

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



VITAMIN B12

Make healthier blood the foundation of a healthier life!

Keep your blood clean and your nerve cells healthy with Vitamin B12. This essential supplement also protects against anemia, which causes persistent fatigue. Extracted from natural food sources, such as amla and beetroot, daily intake of Vitamin B12 can help in the formation of red blood cells.

A lack of vitamin B12 can cause dangerous health complications, such as:

- Memory Loss
- Vision Loss
- Ataxia or Loss of Physical Coordination
- Irreversible Neurological Problems
- Heart Diseases

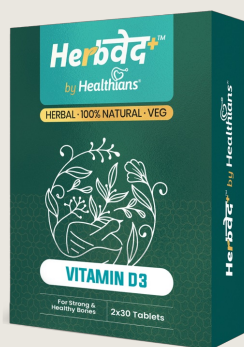
Prevents Osteoporosis & Promotes Bone Health

Prevents Anemia & Promotes Red Blood Cell (RBC) Production

Alleviates Symptoms of Depression

Boosts Heart Health

Prevents Major Birth Defects



VITAMIN D3

Improve bone health with enhanced calcium absorption, the natural way

Make your muscles and bones stronger with VITAMIN D3. Sourced from natural substances, it helps in regulating the absorption of calcium and phosphorus, which help keep your bones strong and enhancing the normal immune system functioning. Vitamin D3 is an essential nutrient that's critical for normal growth and development of bones and teeth, as well as improved resistance against certain diseases.

Remember, a lack of vitamin D3 can cause dangerous health situations.

- Rickets (in children)
- Brittle Bones
- Osteoporosis
- Weakened Bones (in adults)

Strengthens Bones & Muscles

Protects Against Pneumonia & Acute Respiratory Infections

Helps in Reducing Depression

Boosts Heart Health

Aids in Kidney Disease Treatment

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About Healthians Labs

How we control Report Accuracy at Healthians



Quality Control

We follow Quality control to ensure both **precision & accuracy** of patient results.



Machine Data

We save patient's result values **directly from machines** ensuring no manipulations & no fake values.



QR Code

QR Code based authenticity check on all its reports



Calibration

We make use of calibrators to evaluate the **precision & accuracy** of measurement equipment.



Equipment

Our Labs are equipped with state-of-the-art instruments with **cutting edge technology** to provide faster & reliable results.



EQA

Our Labs participate in EQA & show proven accuracy by checking **laboratory performance** through external agency or facility.

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